

Guideline Title: i-gel supraglottic airway device

Guideline Number: GUID-3203-PR016

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

I. **PURPOSE:** To maintain a patent airway with the i-gel supraglottic airway device when direct laryngoscopy and passage of an endotracheal tube is not possible.

II. **DEFINITIONS:** None

III. **DEPARTMENT GUIDELINE:**

A. EQUIPMENT:

<i>i-gel</i>		Pediatric				Adult	
Tube size	1	1.5	2	2.5	3	4	5
Color	pink	blue	grey	white	yellow	Green	orange
Weight	< 5 kg	5-12 kg	10-25 kg	25-35 kg	30-50 kg	50-90 kg	> 90 kg

B. INDICATIONS:

- i. The i-gel is intended for difficult and emergent airway management in the patient who is apneic without a gag reflex.
- ii. If device was previously placed by prehospital personnel and is providing adequate oxygenation and ventilation, do not selectively remove it.

C. CONTRAINDICATIONS:

- i. Responsive patients with an intact gag reflex
- ii. Known esophageal disease
- iii. Caustic substance ingestion
- iv. Patients with laryngectomy / tracheostomy
- v. Complete airway obstruction
- vi. Foreign body aspiration
- vii. Pharyngeal hemorrhage

D. PROCEDURE:

- i. Apply a water-based lubricant to the beveled distal tip and posterior aspect of the tube. Avoid introduction of lubricant in or near the ventilatory openings.

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- ii. Pre-oxygenate patient.
- iii. Position the head (neutral of sniffing position)
- iv. Hold i-gel with dominant hand. With non-dominant hand, gently press chin downward
- v. Insert the igel into the mouth towards the hard palate.
- vi. Advance gently downwards and backwards along the hard palate without exerting excessive force until definitive resistance is felt.
- vii. Attach an end tidal CO2 detector then begin ventilation with a resuscitator bag
- viii. Verify by capnography, visualize rise of chest, auscultate breath sounds and confirm negative epigastric sounds
- ix. If unable to verify placement by capnography, chest rise or breath sounds, remove i-gel and attempt reinsertion.
- x. Secure i-gel with a commercial securing device or tape. Do not cover the proximal opening of the gastric access lumen.
- xi. Once ventilation and oxygenation are confirmed place a gastric tube through the proximal opening of the gastric access channel (the size 1 neonatal does not have a gastric channel) *per GUID3203-PR009 – Gastric Decompression.*
- xii. Continue to ventilate patient with a bag valve device throughout transport.

IV. **DOCUMENTATION:** EMS Charts

V. **REFERENCES:** None